



University of Kentucky Emergency Department	Clinical Pathway - Pediatric Orthopedic
Title/Description: Emergency Department Management of Pediatric Orthopedic Patients	
Purpose: Summarizes the workflow for consulting orthopedics in the management of pediatric patients in the Emergency Department	

Independent Emergency Department Management

- Non-displaced clavicle fractures
- Acromioclavicular separation
- Glenohumeral dislocation – with axillary lateral x-ray demonstrating anatomic reduction
- Non-displaced forearm fracture
- Distal radius buckle fracture
- Non-displaced metacarpal or phalanx fractures of the hand
- Soft tissue injury without bone or joint involvement
- Transient synovitis of the hip – $\leq 1/4$ Kocher criteria (per septic hip algorithm)
- Toddler fracture (non-displaced, spiral tibia fracture)
- Non-displaced metatarsal or phalanx fractures of the foot

Orthopedic Surgery Consult

General rules:

- Any fracture involving the growth plate
- Any displaced fracture
- Any elbow injury
- Any musculoskeletal infection

Specific Injuries:

- Displaced clavicle fractures
- Proximal humerus fractures
- Humeral shaft fractures
- Any elbow injury – normal x-ray w/ effusion, elbow dislocation, medial epicondyle fx, lateral condyle fx, supracondylar humerus fx, radial neck fx
- Displaced forearm fractures
- Displaced metacarpal or phalanx fractures of the hand
- Septic hip – $\geq 2/4$ Kocher criteria (per septic hip algorithm)
- Proximal femur fractures
- Femoral shaft fractures
- Distal femur fractures

- Intra-articular fractures of the knee – tibial spine fx, proximal tibial epiphyseal fx, tibial tubercle fx
- Tibia shaft fractures
- Physeal fractures of the ankle
- Displaced metatarsal or phalanx fractures of the foot

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